[redacted]

Master. [redacted]

PROGRESS SHEET

2024062570844

Patient's Name ... PIMS No ...

Date & Time

13/02/26
1:00pm

C/S/B - [redacted]

Δ - ENTERIC FEVER
Nutritional anemia.
⇒ c/o fever
⇒ c/o loose stools x 2 episodes.
⇒ No c/o lethargy
⇒ No c/o Irritability
⇒ No c/o altered sensorium
⇒ No c/o abdominal pain
⇒ No c/o decrease urine output.
⇒ No c/o altered behaviour
⇒ No c/o Joint pain

O/E:- child was
HR: 100/min
RR: 26/min
SpO2: 98%
BP: 100/70mmHg

S/E:-
CVS: S1 S2 (P)
CNS: No focal neurological deficit
P/A: Soft non tender
Liver is palpable 10.5 cm liver span
⇓
↑ CMR & CH

R/S ⇒ B/L air entry NVBS.

[redacted]

[redacted]

PIMS No .....

Patient's Name .....

Date & Time	
	Plan => To add azithromycin => To continue ceftriaxone => To continue antipyretics => w/f warning.
13/3/26 10:00 am	C/S/B - Dr. NIVYA (AP) Dr. MONICA (SR) Δ = ENTERIC FEVER (uncomplicated) Nutritional Anemia. Child Referred, C/o fever (+) C/o loose stools - 2 episodes C/o Nausea (+) O/E:- Child is Alert / Active HR : 110/min RR : 22/min SpO2 : 98% @ Room Air BP : 100/70 mmHg

[redacted]

[redacted]

Master [redacted]

2024062570844

PROGRESS SHEET

ent's Name . ..... PIMS No .....

te & Time

S/E:-

P/A: Soft, non tender

Liver is palpable 4cm Below RCM

Liver span: 10.5 cm

CVS: S1S2 (+)

CNS: No focal neurological deficit

RS: B/L AE (+)

Plan:-

→ continue ceftriaxone

→ w/f complications.

→ continue symptomatic management.

→ CRP in next prick

[redacted]

[redacted]

AnyScanner